BRIGHTWATER HEALTH PLAN

Evidence Guide: Conservative Treatment

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1. What this guide covers

This guide covers evidence that the member has had conservative treatment before the service: physiotherapy, supervised or home exercise, manual therapy, injections, walking aids and activity modification, and how long the treatment lasted. It is used by the Intake Coordinator who opens a review and the UM Nurse Reviewer who decides it, from the documents sent with a request to the recommendation, and by Provider Services Representatives answering the requesting office.

It comes most often in physiotherapy reports and family physician letters, for MRI of the lumbar or cervical spine and for total knee replacement.

A medicine tried without benefit is failed medication, and the Failed Medication guide covers it. Medicines are part of conservative treatment, but a list of failed medicines gives no length of treatment.

Whether a criterion is met is a separate question, settled by the questionnaire and the service's own medical policy. This guide is about the evidence.

2. How the evidence shows up

- a number of weeks or sessions of physiotherapy, supervised exercise or a home exercise programme
- a course of treatment ended without improvement, or a discharge from physiotherapy
- injections into the knee and how long they helped
- walking aids, a knee brace or activity modification
- conservative management described in a specialist letter

3. What the reviewer checks

These are the checks the reviewer makes, in this order:

1 Find the length of treatment on the questionnaire, in whole weeks, as the Prior Authorisation Rules read it.

2 Read each sentence of this kind for a length of treatment in weeks, months or sessions.

3 Compare the two: a length in a document that differs from the questionnaire's is flagged.

4 Check the treatment was for the region of the request: the back, the neck or the knee.

5 Note whether the treatment ended without improvement or is still going on.

4. Criteria it bears on

It bears on the conservative treatment criterion of the lumbar and cervical spine MRI policies and of the total knee replacement policy. It bears on no criterion of the bariatric surgery or sleep study policies.

5. When the documents disagree

When a document gives a length of treatment other than the questionnaire's, the review flags the disagreement for the reviewer and keeps the questionnaire's answer. A document never lengthens the treatment on the questionnaire, and a blank length is never filled from a letter: the office is asked for it.

6. What to tell the requesting office

Tell the requesting office to give the length of conservative treatment on the questionnaire, in weeks or days, and to send the physiotherapy discharge summary or an attendance record showing when the treatment began and ended. A letter alone never changes the questionnaire.

7. When to refer to a physician

Refer to a Physician Reviewer when the questionnaire's treatment is shorter than the policy asks - with No red flag as its red flag answer, under a spinal MRI policy - or when a flagged disagreement would change the criterion.

Referring does not decide the request: a Physician Reviewer decides it, as the Utilisation Management Policy describes.

8. Recording the evidence

Each sentence's evidence is recorded on the review: the sentence, its document, the kind of evidence, the criterion it bears on and any disagreement flagged.

The review recommends one of the decisions open, and a UM Nurse Reviewer who did not open it decides, as the Utilisation Management Policy describes. Evidence informs the decision; the questionnaire and the medical policy decide the criteria.